

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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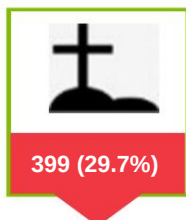
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°046/MVB_29/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (35)	- Ituri (23/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki and Mandima. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 29, 2026
Publication date	June 30, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



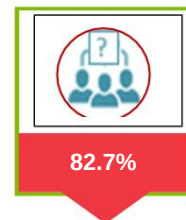
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Seventeen **(26) new confirmed cases, including 8 deaths**, were reported on June 29, 2026, in the provinces of Ituri **(17 cases, including 4 deaths)** and North Kivu **(9 cases, including 4 deaths)**. These confirmed cases originated from the health zones of **Bunia (14), Nizi (2), and Lita (1)** in Ituri, and **Katwa (7), Butembo (1), and Musienene (1)** in North Kivu.

- In parallel, **14 deaths were recorded (including 13 in Ituri and 1 in North Kivu in the Beni health zone)**

among patients with Ebola virus disease in Ebola treatment centers in Ituri province (4 at Ebola Treatment Center, 4 at Ebola Treatment Center, 2 at Ebola Treatment Center, 2 at Ebola Treatment Center, 2 at Ebola Treatment Center, 2 at Ebola Treatment Center, 1 at Ebola Treatment Center).

- Nine (9) patients** with Ebola virus disease in Ituri province (2 at CTE CME, 2 at CTE Nyakunde, 1 at CTE Rwampara, 1 at CTE Elikya) and in North Kivu province (1 at CTE Beni, 1 at CTE Kalunguta and 1 at CTE Katwa) have been declared cured after obtaining negative control tests.

- Start of free healthcare in 4 priority health zones (Bunia, Mongbwalu, Nyankunde and Rwampara) in Ituri.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons. The health zone of

Mongbwalu, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, the Ituri province had already been affected by the 10th Ebola virus (EV) outbreak, which was then raging in North Kivu. The current epidemic, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

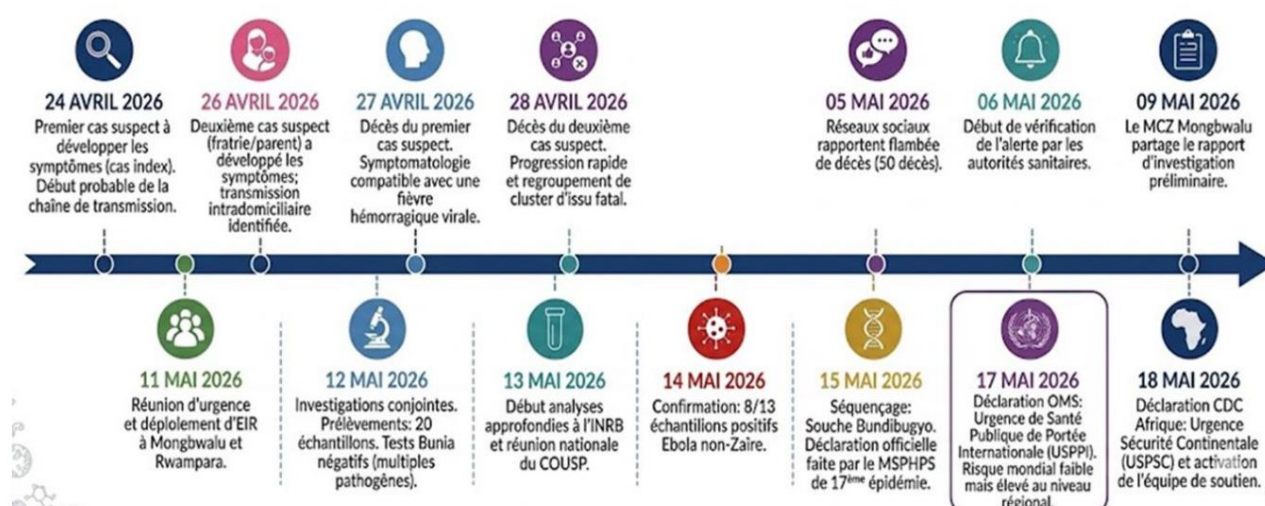


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 29, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 29, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24 hours)
Ituri	1214	335*	27.6%	23 out of 36 (63.8%)	17
North Kivu	116	63*	54.3%	11 out of 34 (32.3%)	9
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1333	399	29.9%	35 out of 104 (33.6%)	26

* 13 deaths in Ituri and 1 death in North Kivu were recorded among patients with Ebola virus disease (previous confirmed cases) in the CTE/CT.

No new health zones were affected during the day on June 29, 2026. The number of affected health zones remains at 35 out of 104 in the three affected provinces. South Kivu province did not record any new cases.

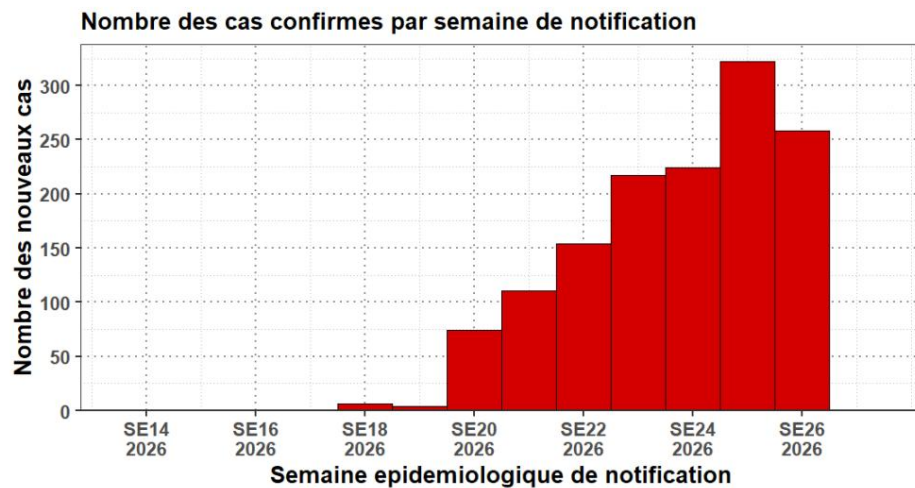
no confirmed cases since the last confirmed case reported on May 26, 2026, suggesting an absence of recent transmission.

3.2 Distribution by health zone (Cumulative total as of 29/06/2026)

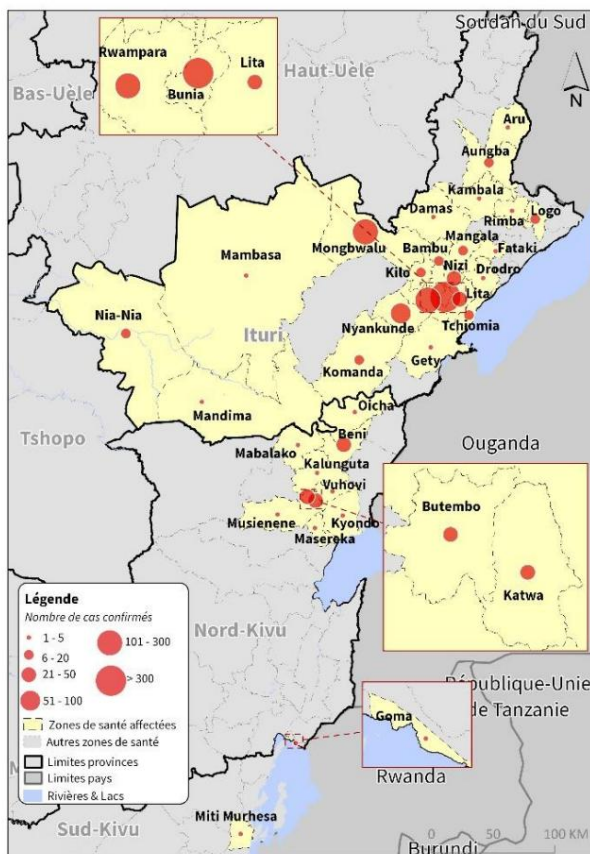
- **Ituri Province (1,214 cases):** Bunia (358), Rwampara (295), Mongbwalu (258), Nyankunde (94), Nizi (40), Lita (29), Mangala (24), Bambu (16), Komanda (16), Tchomia (13), Kilo (9), Logo (7), Aungba (6), Nia-Nia (11), Damascus (5), Rimba (3), Aru (3), Drodoro (3), Mambasa (2), Gety (1), Kambala (1), Fataki (1) and Mandima (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.

- **North Kivu Province (116 cases):** Katwa (45), Butembo (34), Beni (22), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (3), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) contributed to early detection, confirming continued and increased community transmission by week 25. Compared to all the weeks in which cases have been recorded since the beginning of the epidemic, week 25 represents the peak so far, with more than 300 cases per week.



3.4 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 29, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 29, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	1214	335	27.6%
Bunia	358	82	22.9%
Rwampara	295	60	20.3%
Mongbwalu	258	123	47.7%
Nyankunde	94	15	16.0%
Nizi	41	9	22.0%
Lita	29	9	31.0%
Komanda	16	5	31.3%
Bamboo	16	4	25.0%
Kilo	9		11.1%
Mangala	24	1	50.0%
Tchomia	13		15.4%
Damascus	5	12 2 0	0.0%
Aungba	6	2	33.3%
Rimba	3	0	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	7	1	0.0%
Drodro	3	1 0 3	100.0%
Gety		0	0.0%
Kambala			100.0%
Nia-Nia*	11	1	45.5%
Fataki	11 1	5	0.0%
Mandima		0	0.0%
Other areas not yet identified	1 17	0 0	0.0%
NORTH KIVU	116	63	54.3%
Butembo	34	14	41.2%
Katwa	45	27	60.0%
Blessed	22	14	63.6%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	3	1	66.7%
Goma		2	0.0%
Masereka		0	0.0%
Vuhovi	1 1 1	0 1	100.0%
Mabalako		0	0.0%
Musienene	1	2	66.7%
SOUTH KIVU	3 3	1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL	1333	399	29.9%

* The imported cases in Wamba (Hut Uélé Province) are counted in Niania and have been returned to Niania.

Over the past 24 hours, **26 new confirmed cases, including 8 deaths, have been recorded** in the provinces of Ituri (17 cases, including 4 deaths) and North Kivu (9 cases, including 4 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,333 confirmed cases and 399 deaths, expressing an overall case fatality rate of 29.9%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **91.1%** of cases (n=1,214) and **83.9%** of deaths (n=335), representing a case fatality rate of **27.6%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (358 cases), Rwampara (295 cases) and Mongbwalu (258 cases) remain the most affected, accounting for approximately 75.0% of reported cases at the provincial level and 68.3% at the national level. Within these same health zones, moderate case fatality rates were observed in Rwampara (20.3%) and Bunia (22.9%), while Mongbwalu **also** experienced a high case fatality rate of **47.7%** (123 deaths out of 258 cases), suggesting persistent challenges in early intervention or access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported, while the health zones of origin remain to be identified, which could indicate a geographical expansion requiring further investigation.

Severity of the epidemic in North Kivu

A total of 116 confirmed cases and 63 deaths have been reported in North Kivu province since the start of the outbreak, representing a **case fatality rate of 54.3%**. This province presents the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (60.0%), **Beni** (63.6%), and **Oicha** (66.7%). The Katwa health zone has the highest number of reported cases (n=45), followed by the Butembo health zone (n=34) with a case fatality rate of 41.2%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone (which has a small population).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Field visit to the Bunia health zone to assess the expansion capacity of the structures isolation.
- Identification, by the response team (including CREC) supported by technical and financial partners, of the site intended for the development of the CTE in the Bunia health zone.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 29, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		37	0	0	37
New Alerts		474	864	38	1376
Total alerts for today		511	864	38	1413
Alerts investigated		180	862	36	1078
Investigation rate		35.2%	99.8%	94.7%	76.3%
Alerts confirmed	Alive	164	55	0	219
	Deceased	58	32	0	90
Total suspected cases for today		222	87	0	309

- At the end of the day on June 29, 2026, **1341 alerts** were reported across the three provinces, **1047 (75.9%)** were investigated, **309 suspected cases including 87 deaths** were identified (Table 3).

Table 4. Contact tracing of confirmed cases as of June 29, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	9,335	7,825	83.8%
North Kivu*	2,461	1931	78.5%
Total	11,796	9,756	82.7%

* The follow-up rate was calculated only for health zones that submitted reports (completeness: 93.3%). The health zones of Kalunguta (North Kivu) and Kilo (Ituri) did not have data available at the time of report consolidation.

- Exit from all contacts in the province of South Kivu.
- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **988 new contacts** were listed, including **872** in 11 health zones in Ituri province and **116** in 4 health zones in North Kivu province.
- **262 contacts successfully completed their 21-day follow-up**, including **173** in 4 health zones in Ituri province and **89** in 3 health zones in North Kivu province.

Update on checkpoint and point of entry (PoC/PoE) activities

- Activation of the Zunguluka PoC in the Banana AS/Mambasa ZS and continued monitoring of PoE/PoC activated
- **Incident** : The checkpoint under construction by the ICCN in Miala at the northern entrance to the city of Bunia was set on fire on the night of Sunday to Monday, June 29, 2026 by unidentified persons.
- **Ituri** : 1 alert was notified of a lifeless body intercepted at the Mudzibala PoC in the N'zeré centre AS, the response team was contacted for the swab and the body was evacuated to the morgue.

- In North Kivu: Four (4) alerts notified, including 1 live person who was invalidated after secondary screening and 3 lifeless bodies intercepted, were taken to the morgue to carry out the swab while awaiting the EDS teams.

Table 5, Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 29, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)	98.5% (68/69)
Number of people who went through PoE/PoC	95,558	63,827	159,385
testing, % of travelers	96.8%	97.5%	97.1%
screened, % of travelers who washed their	94.2%	97.5%	95.5%
hands, % of travelers made aware of the issue	99.4%	97.5%	98.6%
Number of alerts notified	1	4	5
Number of validated live alerts	0	0	0
Number of lifeless bodies intercepted today	1	3	4
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	0.0%	0.0%	0.0%
% of lifeless bodies swabed	100.0%	100.0%	100.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 64 samples collected and analyzed (**positivity 26.6%; n=17**).
- **North Kivu** : 97 samples received, of which 77 were analyzed (**positivity 11.7%; n=9**).
- **South Kivu** : 0 samples were collected and analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: 6/17 households were identified and decontaminated around confirmed cases; 8 health establishments identified and decontaminated around confirmed cases in the health zones of Bunia, Rwampara, Mongbwalu, Komanda; 9 decontaminations of public and specific places: Sokimo office, DGPI, COUSP; 26/29 health assessments carried out with 2 postponements; IPC/WASH assessment of 6 health establishments in the health zones of Mongbwalu, Bunia, Lita, Nyankunde, Rwampara: Katoto Health Center: Ushindi Health Center 38.7%, Maranatha Health Center 45.2%, Komanda Health Center 76%, Health First Health Center 34%, Dispensary, Afia Bora 30.6%, Unibu Hospital 71%; Decontamination of 25 means of transport (ambulance, hearses) and briefing of 33 service providers on Ebola Virus Disease; 93 PPLs trained on standard PCI precautions in the ZS of Bunia (50), Nizi (18), Mambasa (25) as well as the allocation of 3 household kits in the ZS Mongbwalu and Nia-Nia.

North Kivu : 18 EDS (Emergency Response Systems) were carried out out of 32 death alerts, representing a coverage of 56.2%. Although the average response time of four hours is in line with operational standards and 31 SWABs (Swabs) were performed.

4.5 Holistic care

- In parallel, 13 deaths were recorded among patients with Ebola virus disease in the Ebola treatment centers of the province of Ituri (4 at CTE Elikya, 4 at CTE CME, 2 at CTE Mongbwalu, 2 at CTE Nyankunde and 1 at the CTE ISTM) and 1 death at the CTE Beni in North Kivu. Thus, the cumulative number of recovered patients is **189, including 176 in Ituri, 11 in North Kivu and 2 in South Kivu.**
- By the end of the day on June 29, 2026, 691 hot meals had been served, including 163 to the cases confirmed, 134 to suspected cases, 340 to PPLs and 54 to accompanying persons, Twenty (7) dry rations were also distributed.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 29, 2026

Indicator	Ituri ¹	North Kivu	South Kivu	Together
Patients in bed (Day -1)	449	138	23	610
Total admissions (24h)	68	11	3	82
Exits (24h)				
Deceased (Suspects/Confidence)	17	1	0	18
No case	24	3	6	33
Healed	6	3	0	9
Escapees (Suspects/Confessions)	3	2	0	5
Transferred to the HGR	0	0	0	0
Total outflows	50	7	6	63
Patients in isolation (End of Day)	451	135	23	609
including confirmed (NC+AC)	200	16	0	216
including suspects	254	119	23	396
Overall occupancy rate	102%	125%	51.1%	92.7%

1 Several transit centers (TCs) have been made operational in the different health zones of the Ituri province, bringing the reception capacity up.

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 29, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (66.7%)	1 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	79 (61.7%)	9 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	11 (55.0%)	5 (100.0%)	1 (100.0%)
Suspected cases under follow-up that have received interventions SMSPS	88 (63.3%)	12 (100.0%)	1 (100.0%)
Laboratory results have been announced.	14	23	1
Including positive ones	0	1	0
Number of children separated in daycare who benefited from interventions SMSPS	0	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	3	0
Number of accompanying persons at the CTE who benefited from interventions SMSPS	56	2	0
Number of PPLs who benefited from SMSPS interventions	11	2	0
Number of household and community members who benefited from SMSPS interventions	31	1	0
Number of EDS supported	16	3	0

4.6 Continuity of care

- Launch of free healthcare in the 4 priority health zones (Bunia, Mongbwalu, Nyankunde and Rwampara) in the Ituri province.
- Provision of essential medicines for PMA and PCA in other health zones.
- Psycho-social support for affected PPLs and their dependents.
- Support for non-cases (children, mothers and other vulnerable individuals) in continuity of care actions services

4.7 Risk communication and community engagement

Ituri: 678 social mobilization activities including 641 (community dialogues and mass awareness) reaching 11,308 (6,639 men and 4,669 women) on preventive measures for Ebola virus disease, community-based surveillance, care, decontamination and EDS in the health zones of Mongbwalu Nyankunde, Nizi, Rwampara, Nia-Nia, Aungba, Bambu, Aru; 50 educational talks and home visits organized by 1,241 Recos, 10,449 households were reached and 8,902 people reached (4,236 men and 4,666 women) in the health zones of Bunia, Nyankunde; Briefing of 5 providers on community-based surveillance, CREC with practical exercises on the organisation of home visits, facilitation of educational talks in their respective environments and the use of tools (community definition of cases, contact registration form, contact tracing form, and home visit form); 18 broadcasts of spots and messages on preventive measures for Ebola Virus Disease.

Community feedback: Why hasn't the government found a vaccine yet? Some sick people are isolated in their homes because they aren't recovering in hospitals; why aren't people who handle money infected? We are facing other threats, including the response...

must take into account, in order to properly manage the Ebola epidemic, population displacement, the phenomenon of street children and insecurity; Our police officers who work with you don't even have disinfectant, are you protecting them as you protect yourselves?

Support for the pillars of the

response : Surveillance: Listing of contacts around 3 living cases and 2 deceased cases, Acceptance of the 21-day observation period and the decontamination of their home; Support for the investigation and listing of contacts around 1 case in Rwampara, 1 in Dele, 1 in Ngona, 1 in Tchungu, 2 in Rwampara, 1 in Sukisa; Support for listing 51 people around a case of death in the TSERE site.

PCI : Preparation of two families for participation in the EDS following a community death (Nizi health zone and Rwampara health zone). Result: Decontamination of 17 households, including 1 in Hoho and 15 at the Tshere displacement site, and 1 in Shari.

North Kivu: 2,962 people were reached through community outreach, including (58% women and 42% men) and 641 people who previously refused public health initiatives who were transferred to care. While female representation is good, men remain underrepresented even though they constitute the most receptive audience to the identified rumors. An additional 819 people were reached through public spaces and places of worship.

Although this volume remains modest, these channels have strong potential to specifically reach adult men and should therefore be intensified; 32,801 people reached in total during the day, confirming a significant reach; 59 community alerts received, representing an awareness/alert conversion rate of 0.18%: However, a spatial analysis remains necessary to identify any potential areas of epidemiological silence.

Feedback collected and clarified : While the responsiveness is excellent, it is clear that this volume is very low considering the 31,982 people reached, indicating a lack of active listening that urgently needs to be addressed; 713 leaders were involved in the response with a commitment to mobilize the socio-professional groups in their communities to adopt hand hygiene measures; 20 community actors in the Karisimbi health zone received continuing training, including teachers, community health workers, traditional healers, religious leaders, and pharmacists. Thanks to this diversity of profiles, the response has acquired credible and strategic messengers to counter rumors.

4.8, Logistics

Ituri

- Supply of medicines and IPC inputs (Village Reach donation) to the Nyankunde Ebola Treatment Centers, CME, Rwampara, Elihya, HGR Bunia and Mongwalu;
- Logistical supply of PCI, PECH, Surveillance, and EDS inputs to health zones
Rwampara, Komanda, Nizi and Beni: Logistics/Ambulance Transport Pillar, Security and Safety Pillar
(Total weight: 5567 for a total of 157 packages).

North Kivu

- Continued delivery of IPC inputs to response teams.

4.9, Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Briefing of 113 actors (26 women and 87 men), including ambulance workers, nurses, hygiene workers and Red Cross volunteers; Continuation of community awareness-raising and planning of training for zonal PSEA focal points; Awareness-raising of 38 people (24 women and 14 men).



Unloading ceremony for the first Ebola survivor in the Beni health zone of North Kivu

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of the CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3. Contact tracking performance not optimal (82.5% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 case)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any possible duplicates and reclassify. or specify the health zone and status (deceased, recovered, active, etc.). Active verification on the ground.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 cases) HCW (including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8 Stock of non-sample samples further analysis (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities

9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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